

CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: JB Period: July 2 to September 30, 2024 Report Date: October 1, 2024 Coverage: Bed: 1772/2174h (81.5%)
Chair: 1474/2174h (67.8%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 91 days from Jul 02 to Sep 30



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
291	624h	Very High Heart Rate	3	100%

Major Findings: Sustained very high HR (avg 94, peak 148)

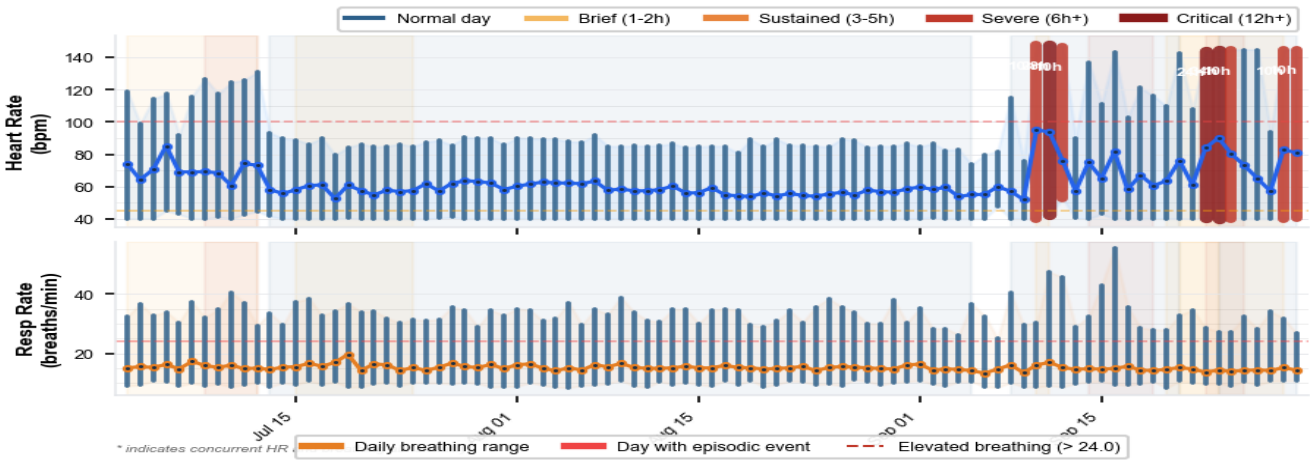
#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Very High Heart Rate	145 bpm	66h	24h	1	74 bpm	Sep 21 to Sep 29
2	Very High Heart Rate	148 bpm	35h	10h	2	94 bpm	Sep 10 to Sep 11
3	High Heart Rate	144 bpm	26h	7h	2	68 bpm	Sep 14 to Sep 19
4	High Heart Rate	127 bpm	7h	2h	1	69 bpm	Jul 08 to Jul 12
5	High Heart Rate	145 bpm	7h	2h	2	82 bpm	Sep 23 to Sep 26
6	Low Heart Rate	40 bpm	339h	8h	3	58 bpm	Jul 13 to Sep 05
7	High Breathing (brief)	55 brpm	20h	4h	<1	37 brpm	Jul 07 to Sep 22

+ 5 additional condition(s) across Elevated Breathing, Elevated Heart Rate, Low Heart Rate; details in monitoring data.

The P5 to P95 heart rate spread was 54 bpm (43 to 97), indicating significant cardiac variability.

Clinical Pattern Observations:

- Sustained finding:** 24h continuous Very High Heart Rate beginning Sep 23.
- High variability:** HR spread 54 bpm (42 to 96).
- Clustered pattern:** Episodic events on 8 of 91 days (8%).



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.